PEARSON EDEXCEL GCSE (9–1) HISTORY • PAPER 1 OPTION 11

MEDIEVAL MEDICINE IN BRITAIN (C1250–C1500)

Supernatural Orthodoxy, The Galenic Monopoly & The Black Death



HISTORICAL OVERVIEW • C1250–C1500

In medieval Britain, illness was understood through Catholic theological dogma and ancient classical authority. The Church held an institutional monopoly over learning, scriptoria, and universities, declaring Claudius Galen infallible. Disease was interpreted as divine retribution for sin, celestial alignment, or humoural imbalance, while human dissection remained strictly prohibited and questioning doctrine was condemned as heresy.

When the catastrophic Black Death struck in 1348, killing over a third of the population, a society bound to the Four Humours and miasma theory stood powerless. From flagellants in city streets to herbal remedies, prayer, and bloodletting, this volume charts an age where religious devotion and Galenic tradition defined the human struggle against disease.

SPECIFICATION & ENQUIRY DIRECTORY

Pearson Edexcel GCSE (9–1) History

ENQUIRY 01 pp. 3–4
Supernatural & Religious Explanations of Disease
KEY QUESTION: *"Why did the Catholic Church hold an unshakeable monopoly over beliefs?"*

- **Ecclesiastical Monopoly:** Catholic Church controlled education, universities, and medical licensing, enforcing doctrine that illness was divine retribution for sin.
- **Astrological Alignment:** 1345 planetary conjunction of Saturn, Jupiter, and Mars in Aquarius blamed for corrupting air and bodily humours.
- **Galenic Teleology:** Galen's belief in a single divine Creator adopted as Church orthodoxy; questioning classical doctrine condemned as heresy.

ENQUIRY 02 pp. 5–6
Rational Explanations: Galen & The Four Humours
KEY QUESTION: *"How did the Four Humours provide a complete explanation for illness?"*

- **Humoural Equilibrium:** Health required balancing Blood, Phlegm, Yellow Bile, and Black Bile, linked to seasons and elemental qualities.
- **Theory of Opposites:** Galen extended Hippocrates by treating humoural excess with opposing qualities (e.g. cold phlegm treated with hot peppers).
- **Miasma Theory:** Rotting organic matter and swamp vapors believed to produce foul airborne miasma that disrupted internal humoural balance.

ENQUIRY 03 pp. 7–8
Approaches to Treatment & Prevention: Bleeding & Purging
KEY QUESTION: *"Why did standard treatments frequently endanger the lives of patients?"*

- **Phlebotomy & Purging:** Bloodletting via vein incision, cupping, or leeches guided by the Zodiac Man (Homo Signorum); violent emetics and laxatives.
- **Religious Penitence:** Confession, pilgrimages to holy shrines, prayer to patron saints, and wearing religious amulets to appease God.
- **Miasma Countermeasures:** Carrying pomanders stuffed with sweet herbs, burning aromatic incense, and ringing church bells to clear foul air.

ENQUIRY 04 pp. 9–10
Medical Care Providers & Monastic Hospitals: Care Not Cure
KEY QUESTION: *"How did social hierarchy and the Church shape medical care providers?"*

- **Physicians & Barber-Surgeons:** University-trained physicians examined urine charts for the rich; barber-surgeons performed bloodletting and minor surgery.
- **Monastic Hospitals:** Run by monks and nuns offering warmth, shelter, and prayer ("care not cure"); infectious and terminal patients strictly excluded.
- **Wise Women & Apothecaries:** Local herbalists provided traditional folk remedies in villages; apothecaries mixed herbal recipes and charms.

ENQUIRY 05 pp. 11–12
Case Study: Dealing with the Black Death (1348–1349)
KEY QUESTION: *"Why were medieval communities and authorities powerless to stop plague?"*

- **Arrival & Catastrophic Toll:** Plague landed at Melcombe Regis in 1348, wiping out 30% to 45% of England's total population within eighteen months.
- **Desperate Treatments:** Strapping live chickens to buboes, lancing swellings with hot irons, and public self-flagellation in city streets.
- **Civic Failure:** Quarantine attempts failed; authorities dug mass plague pits, shut city gates, but lacked understanding of flea or rat vectors.

THEMATIC MATRIX pp. 2–11
Thematic Matrix: Longitudinal Factors & Synthesis
KEY QUESTION: *"Which historical mechanisms dominated medicine across the Middle Ages?"*

- **The Catholic Church:** Maintained an absolute institutional monopoly on knowledge; preserved Galen's texts while strictly forbidding human dissection.
- **Continuity & Orthodoxy:** Humoural balancing and miasma theory created complete intellectual continuity, preventing any change in clinical therapy.
- **Government & War:** Weak royal government played minimal public health role beyond emergency Black Death quarantines and minor town ordinances.

SPECIFICATION PAPER	SECTION B QUESTION ARCHITECTURE	ASSESSMENT WEIGHTING
Paper 1 Option 11 • Thematic Study	Q3: Similarity/Diff [4m] • Q4: Causation [12m] • Q5/Q6: Essay [	36 Raw Marks + 4 SPaG • 70% of Paper 1 T

ENQUIRY 01 • PART I

Supernatural & Religious Explanations of Disease (c1250–c1500)

KEY ENQUIRY QUESTION *Why did the medieval Catholic Church hold an unshakeable monopoly over beliefs about the causes of disease?*

[1.1] Divine Retribution and Original Sin

"How did church scriptoria prevent new medical ideas from spreading?"

Throughout the medieval era between 1250 and 1500, the Roman Catholic Church was the supreme authority governing everyday life, thought, and morality across Western Europe. Ordinary peasants and learned scholars alike believed that disease was not merely a physical malfunction of the body, but a direct manifestation of God's will. Illness was frequently understood as divine punishment for individual transgression or collective sin, recalling the biblical plagues sent upon Egypt. Suffering was viewed as an opportunity for spiritual purgation: enduring sickness patiently on earth could shorten an individual's time in Purgatory. Consequently, whenever epidemics broke out, communities turned first to penitential processions, communal fasting, and confession before seeking earthly herbal remedies.

[1.2] Moral Taint: Leprosy and Social Exclusion

"Why was leprosy viewed as visible proof of divine judgment?"

Because disease was viewed as fundamentally supernatural in origin, treatments were predominantly religious and spiritual. The sick undertook arduous pilgrimages to holy shrines —most famously the tomb of Archbishop Thomas Becket at Canterbury Cathedral or Our Lady of Walsingham. Pilgrims prayed before sacred reliquaries containing bone fragments, garments, or vials of saintly blood, believing that physical contact or proximity could channel miraculous divine healing. Wealthy families paid chantries to sing perpetual intercessory masses for their sick relatives, while communities carried holy relics in solemn street processions to ward off pestilence.

ENQUIRY 01 • PART II

TURNING POINTS & HISTORICAL CONSEQUENCES

Teleology & The Papal Monopoly on Knowledge

DECISIVE HISTORICAL MECHANISM Questioning Galen became synonymous with heresy against God, freezing anatomical progress for over 1,300 years.

[1.3] Astrological Influences and the Zodiac Man

"How did the Church reconcile astrology with Christian doctrine?"

Few diseases demonstrated the fusion of physical malady and spiritual judgment as vividly as leprosy. Lepers were believed to bear an outward physical sign of internal moral corruption and sinful lust, though some religious orders also viewed them with Christian pity as sharing Christ's earthly sufferings. Once diagnosed by a priest or bishop, lepers were legally cast out of society through a formal burial rite: earth was cast upon their feet to symbolise that they were dead to the world. They were compelled to reside in isolated leper houses (lazar houses) outside town boundaries, wear distinctive grey cloaks, and rattle wooden clappers to warn healthy travellers away.

[1.4] Spiritual Intercession, Relics, and Miracle Cures

"Why did sick people turn to pilgrimage shrines rather than doctors?"

Alongside divine intervention, medieval society relied heavily on astrology to explain the unpredictable timing and severity of disease. Drawing upon Arab scholarship and classical cosmological texts, medieval physicians believed that the macrocosm of celestial planets directly influenced the microcosm of the human anatomy. The position of the Moon, Mars, and Saturn was thought to govern specific bodily organs and fluids. Before administering treatments such as phlebotomy (bloodletting), purging, or compounding herbal cordials, physicians consulted complex almanacs and zodiac diagrams. Operating while the Moon was in an unfavorable sign was believed to bring fatal hemorrhaging or exacerbate corrupt internal humours.

[1.5] Censorship, Roger Bacon, and the Church Ban on Dissection

"Why did church enforcement of Galen prevent medical progress?"

While the Catholic Church preserved ancient medical manuscripts in monastic scriptoria, it strictly policed intellectual inquiry and fiercely suppressed scientific heresy. Medical doctrine was required to conform absolutely to Christian scripture; because Galen taught that the human body was created by a purposeful divine designer (teleology), church authorities canonised his anatomical errors as indisputable truth. Dissection of human cadavers was strictly regulated or banned outright across Catholic Christendom, viewed as a sacrilegious mutilation of bodies created in the image of God destined for physical resurrection on the Last Day. Scholars who challenged this rigid intellectual monopoly faced severe ecclesiastical retribution. In the 1260s, Franciscan friar and philosopher Roger Bacon was imprisoned after urging fellow scholars to abandon blind deference to ancient dogma in favour of empirical observation, scientific experiment, and direct sensory investigation of the natural world. Consequently, the Church's iron grip over university medical faculties ensured that medieval medical theory remained intellectually frozen for over two centuries.

ENQUIRY 02 • PART I

Rational Explanations: Hippocrates, Galen & The Four Humours (c1250–c1500)

KEY ENQUIRY QUESTION *How did the Theory of the Four Humours provide a logical, complete explanation for every illness in medieval England?*

[2.1] Hippocratic Medicine and the Four Humours

"How did Hippocrates separate medicine from superstition?"

Alongside religious beliefs, medieval physicians possessed a systematic rational theory inherited from Ancient Greece and Rome. Hippocrates (c.460–c.370 BC) argued that disease was caused by natural, physical imbalances within the body rather than malicious demons. He established the Theory of the Four Humours: the body contained four vital liquids—Blood (warm and wet), Phlegm (cold and wet), Black Bile (cold and dry), and Yellow Bile (warm and dry). Each humour corresponded to one of the four earthly elements (air, water, earth, fire) and changed with the seasons. A healthy individual possessed balanced humours; illness arose when an excess or deficiency occurred.

[2.2] Galen's Theory of Opposites and Humoral Balance

"Why did humoral theory seem logical to medieval people?"

In the second century AD, the Greek physician Galen working in Rome greatly expanded Hippocratic humoralism by creating the Theory of Opposites (contraria contrariis curantur). While Hippocrates had emphasised clinical observation and gentle dietary regulation to allow nature to restore humoral balance, Galen argued that physicians must actively intervene to counteract excess humours with their exact clinical opposite. If a patient suffered from a fever—believed to be caused by an excess of blood or yellow bile, characterised as hot and dry—the physician must prescribe cold, wet remedies such as cucumber, cooling herbs, or cold baths. Conversely, a winter phlegmatic chest chill (cold and wet) required hot, dry treatments such as pepper, wine, and warm blankets. Galen also developed an intricate system of bodily anatomy based largely on dissecting pigs, apes, and dogs. Because human dissection was socially taboo in ancient Rome, Galen assumed human anatomy was identical to animal structures, erroneously claiming that the human jaw consisted of two bones, the liver had five lobes, and blood was manufactured in the liver and absorbed by the body. Despite these errors, Galen's logical system dominated medical practice for over 1,300 years.

ENQUIRY 02 • PART II

TURNING POINTS & HISTORICAL CONSEQUENCES

The Humoural Axis: Balance, Phlebotomy & Opposites

DECISIVE HISTORICAL MECHANISM Humoural medicine was rational because it sought natural biological causes rather than demonology, yet remained medically ineffective.

[2.3] Clinical Observation: Uroscopy and Pulse Taking

"How did the Theory of Opposites provide practical treatment rules?"

Trained medieval physicians practised clinical observation through non-invasive physical examination. The two primary diagnostics were taking the patient's pulse and inspecting their urine (uroscopy). Physicians collected urine in a glass flask called a jordan and compared its colour, density, smell, and even taste against printed urine wheels. A dark, thick urine indicated burning yellow bile, whereas thin, pale urine pointed to excessive phlegm. By combining uroscopy with the patient's astrological chart and temperament, the physician determined which humour required expelling.

[2.4] Galen, Teleology, and the Church's Monopoly

"Why did uroscopy give physicians great diagnostic authority?"

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[2.5] Miasma Theory and Environmental Health

"Why did rational physicians link bad smells directly to epidemics?"

In the second century AD, the Roman-Greek physician Galen expanded Hippocrates' work. Galen devised the Theory of Opposites: if a disease was caused by an excess of cold and wet phlegm, it should be treated with hot and dry remedies, such as eating mustard, dry toast, or pepper. Crucially for medieval survival, Galen was a monotheist who argued that every organ in the human body was designed by a single Creator for a divine purpose (teleology). Because Galen's ideas fitted Christian scripture, the Catholic Church embraced his writings as absolute, immutable truth. For over a millennium, questioning Galen was condemned as heresy.

ENQUIRY 03 • PART I

PEARSON EDEXCEL GCSE (9–1) HISTORY • PAPER 1 (1HI0/11)

Approaches to Prevention & Treatment: Rituals, Bleeding & Purging (c1250–c1500)

KEY ENQUIRY QUESTION *Why did standard medieval medical treatments frequently endanger the lives of the patients they were meant to cure?*

[3.1] Spiritual Remedies: Prayer, Fasting, and Miraculous Intercession

"Why did ordinary people consider prayer more vital than medicine?"

Before any physical medicine was administered, medieval patients sought spiritual pardon. Sickness was understood as a trial sent by God, meaning that medical remedies could not succeed without divine mercy. The primary treatments were therefore religious: saying special prayers such as the Ave Maria and Paternoster, paying for chanting masses, fasting on bread and water, and lighting consecrated candles at local parish altars. Affluent barons gave money to monasteries to pray for their souls, while sufferers of chronic ailments undertook arduous pilgrimages to sacred shrines, such as Canterbury Cathedral to touch the shrine of St Thomas Becket or Walsingham to venerate the Virgin Mary. Pilgrims purchased pewter badges and ampullae containing 'holy water' mixed with traces of saintly blood to rub on diseased limbs. In times of severe epidemics, flagellants marched through towns whipping themselves with knotted iron-tipped scourges in public penance to appease God's anger.

[3.2] Phlebotomy: Restoring Balance through Bloodletting

"Why was bloodletting the most universal physical treatment?"

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ENQUIRY 03 • PART II

TURNING POINTS & HISTORICAL CONSEQUENCES

Regimen Sanitatis: Preventive Hygiene & The Six Non-Naturals

DECISIVE HISTORICAL MECHANISM Medieval people actively cared about hygiene; sickness was prevented by keeping the internal humours and external environment pure.

[3.3] Purging, Emetics, and Clysters: Evacuating Toxic Humours

"How was purging believed to restore bodily equilibrium?"

When phlebotomy was deemed insufficient or when the digestive system was implicated in humoral corruption, physicians and barber-surgeons employed systematic purging to evacuate poisons from the stomach and intestines. Patients were administered powerful herbal emetics formulated from hellebore or antimony to induce violent vomiting. Alternatively, laxatives made from aloe, linseed, senna, or scammonium were prescribed to flush the bowels. For stubborn ailments, practitioners administered a clyster—an enema injected into the rectum using a pewter or copper syringe attached to an animal bladder. Clyster solutions consisted of warm water infused with camomile, crushed marsh mallows, salt, and melted boar's lard. Purging was so violently exhausting that patients frequently faint from dehydration, but medieval healers believed this physical trauma proved that corrupted, malignant humours were successfully leaving the patient's system.

[3.4] Herbal Pharmacy: Simples, Electuaries, and Theriac

"Why did herbal recipes remain popular across all social classes?"

Because disease was viewed as a physical surplus of corrupt humours, the most common clinical treatments aimed to expel excess fluids. Phlebotomy (bloodletting) was the universal remedy for fevers, inflammation, and headaches. Barber-surgeons performed phlebotomy through three main methods: opening a vein with a fleam or lancet (venesection), applying heated glass cups to create a vacuum over scratched skin (cupping), or applying blood-sucking leeches to delicate areas. Bloodletting was so routine that monastic orders scheduled communal bleeding sessions (minutio) six times a year, granting monks days of rest and richer food while recovering from weakness.

[3.5] Surgical Interventions and Preventative Regimens

"How did wealthy barons attempt to prevent sickness before it struck?"

When phlebotomy was deemed insufficient or when the digestive system was implicated in humoral corruption, physicians and barber-surgeons employed systematic purging to evacuate poisons from the stomach and intestines. Patients were administered powerful herbal emetics formulated from hellebore or antimony to induce violent vomiting. Alternatively, laxatives made from aloe, linseed, senna, or scammonium were prescribed to flush the bowels. For stubborn ailments, practitioners administered a clyster—an enema injected into the rectum using a pewter or copper syringe attached to an animal bladder. Clyster solutions consisted of warm water infused with camomile, crushed marsh mallows, salt, and melted boar's lard. Purging was so violently exhausting that patients frequently faint from dehydration, but medieval healers believed this physical trauma proved that corrupted, malignant humours were successfully leaving the patient's system.

When bloodletting was inappropriate, medieval patients turned to herbal pharmacy provided by apothecaries, monks, or local wise women. Herbal remedies were prepared as "simples" (single herbs) or complex compounds called "electuaries" (medicinal powders blended with clarified honey or sugar syrup). The most famous and costly remedy was Theriac (or Venice Treacle), an ancient compound containing over sixty ingredients, including dried viper flesh, opium, cinnamon, and saffron. Believed to purge poison from the heart, Theriac was administered against everything from venomous snakebites to the Black Death.

ENQUIRY 04 • PART I

Medical Care Providers & Monastic Hospitals: 'Care Not Cure' (c1250–c1500)

KEY ENQUIRY QUESTION *How did social hierarchy and the Catholic Church shape who provided medical care in medieval England?*

[4.1] University-Trained Physicians: Theory Over Practice

"Why were university physicians accessible only to the ultra-wealthy?"

Medical care in medieval Britain was strictly stratified by social class and wealth. At the top of the hierarchy were university-trained physicians. Educated at Oxford, Cambridge, Paris, or Montpellier, physicians were scholastic scholars who rarely touched patients directly. They studied ancient Greek and Arab texts in Latin, diagnosing illnesses by reading charts, inspecting urine flasks, and casting horoscopes. Because their training took up to ten years, their fees were exorbitant, limiting their services to royalty, nobility, and wealthy merchants.

[4.2] Guild Apothecaries and Spicers: Retail Medicine

"How did apothecaries provide accessible medicines to townspeople?"

For urban townspeople and merchants who could not afford a university physician, the local apothecary shop was the first port of call. Apothecaries were organized into trading guilds (often alongside pepperers and grocers) and trained through rigorous seven-year manual apprenticeships. They stocked hundreds of dried roots, leaves, minerals, animal fats, and imported spices from Mediterranean trade networks. Unlike physicians who charged high diagnostic consultation fees and wrote prescriptions in academic Latin, apothecaries diagnosed customers directly across the shop counter and compounded customized ointments, cough syrups, and digestive electuaries on the spot. While physicians viewed apothecaries with professional snobbery as mere tradesmen motivated by commerce, ordinary citizens valued their practical pharmacology, accessible advice, and affordable pricing.

ENQUIRY 04 • PART II

TURNING POINTS & HISTORICAL CONSEQUENCES

'Care Not Cure': The Monastic Hospital Philosophy

DECISIVE HISTORICAL MECHANISM "Care Not Cure" defines the institutional purpose of all 500+ medieval English monastic hospitals.

[4.3] Barber-Surgeons and Craft Medicine

"Why were barber-surgeons viewed as lower-status craftsmen?"

For urban townspeople and merchants who could not afford a university physician, the local apothecary shop was the first port of call. Apothecaries were organized into trading guilds (often alongside pepperers and grocers) and trained through rigorous seven-year manual apprenticeships. They stocked hundreds of dried roots, leaves, minerals, animal fats, and imported spices from Mediterranean trade networks. Unlike physicians who charged high diagnostic consultation fees and wrote prescriptions in academic Latin, apothecaries diagnosed customers directly across the shop counter and compounded customized ointments, cough syrups, and digestive electuaries on the spot. While physicians viewed apothecaries with professional snobbery as mere tradesmen motivated by commerce, ordinary citizens valued their practical pharmacology, accessible advice, and affordable pricing.

[4.4] Monastic Hospitals: 'Care Not Cure'

"Why did monastic hospitals deliberately avoid medical treatment?"

By 1500, England possessed over 700 hospitals, virtually all founded, funded, and operated by monastic religious orders. Famous examples included St Bartholomew's (1123) and St Thomas's (1215) in London. These institutions were not medical treatment centres in the modern sense: they admitted the elderly, the homeless, pilgrims, and the chronically infirm, but specifically rejected infectious, incurable, or dying patients to prevent contagion. The guiding ethos was strictly "care not cure": providing clean bedding, warm fires, simple nourishment, and daily religious confession to comfort the patient's soul for eternity.

[4.5] Domestic Healers and Wise Women

"Why did the vast majority of sick people rely on female family members?"

Far lower in social standing were barber-surgeons and apothecaries, organised into urban trade guilds. Barber-surgeons had no university education; they learned their manual trade through seven-year apprenticeships. Armed with razors, lancets, and saws, they cut hair, shaved beards, pulled teeth, dressed wounds, and performed bloodletting. Apothecaries mixed and sold herbal remedies, spices, and electuaries. Although cheaper than physicians, their potions were still beyond the means of ordinary agricultural labourers.

ENQUIRY 05 • PART I

Case Study: Dealing with the Black Death (1348–1349)

KEY ENQUIRY QUESTION *Why were medieval communities, doctors, and civic authorities completely powerless to stop the Black Death?*

[5.1] Arrival and Clinical Symptoms of the Pestilence

"How did trade routes unwittingly bring the plague to England?"

In June 1348, the Black Death (Yersinia pestis) arrived in England aboard merchant trading vessels landing at the Dorset port of Melcombe Regis. The disease presented primarily in two terrifying forms: bubonic plague, spread by fleas living on black rats, causing high fever, vomiting, and agonizing dark swellings (buboes) in the groin and armpits; and pneumonic plague, spread through airborne respiratory droplets, which attacked the lungs and killed victims within forty-eight hours with painful coughing of blood. Between 1348 and 1350, the pandemic wiped out between one-third and one-half of the English population.

[5.2] Clinical Symptoms: Bubonic and Pneumonic Terror

"Why was the pneumonic strain far deadlier than the bubonic form?"

The Black Death attacked the human body with terrifying speed in two distinct epidemiological forms. The bubonic strain, transmitted by fleas from infected black rats, attacked the lymphatic system: victims suffered violent shivering, vomiting, high fevers, and the eruption of agonizing, foul-smelling swellings called buboes in the groin, armpits, and neck, which grew to the size of apples and turned black as internal hemorrhaging occurred. If the buboes failed to burst, victims died within three to five days. Even more lethal was the pneumonic strain, which spread directly through airborne coughs and sneezes into the respiratory tract. Patients spat blood, experienced suffocating chest pain, and perished within twenty-four hours without ever developing buboes. The mortality rate of pneumonic plague approached nearly 100%, and its rapid airborne transmission terrorized doctors and priests, who fled infected sickbeds in panic.

ENQUIRY 05 • PART II

TURNING POINTS & HISTORICAL CONSEQUENCES

Astrology, Miasma & Contagion: The 1348 Conjunction

DECISIVE HISTORICAL MECHANISM Medieval authorities attempted practical public health measures, but lacked biological knowledge of flea vectors.

[5.3] Medieval Explanations: Wrath of God and Planetary Alignment

"Why were medical treatments completely useless against the plague?"

The Black Death attacked the human body with terrifying speed in two distinct epidemiological forms. The bubonic strain, transmitted by fleas from infected black rats, attacked the lymphatic system: victims suffered violent shivering, vomiting, high fevers, and the eruption of agonizing, foul-smelling swellings called buboes in the groin, armpits, and neck, which grew to the size of apples and turned black as internal hemorrhaging occurred. If the buboes failed to burst, victims died within three to five days. Even more lethal was the pneumonic strain, which spread directly through airborne coughs and sneezes into the respiratory tract. Patients spat blood, experienced suffocating chest pain, and perished within twenty-four hours without ever developing buboes. The mortality rate of pneumonic plague approached nearly 100%, and its rapid airborne transmission terrorized doctors and priests, who fled infected sickbeds in panic.

[5.4] Desperate Approaches to Treatment

"How did belief in miasma shape royal prevention orders?"

Because contemporaries possessed no knowledge of microscopic bacteria, they explained the catastrophe through established religious and rational frameworks. Church leaders proclaimed the pestilence was God's righteous punishment for human sins such as pride, swearing, and immodest dress. Scholars at the University of Paris pointed to celestial mechanics, reporting that a rare conjunction of Saturn, Jupiter, and Mars in 1345 had drawn up poisonous vapours from the sea. Other physicians blamed corrupted miasmas arising from unburied corpses, stinking latrines, and filthy city ditches.

[5.5] Attempts at Prevention, Mass Graves, and Social Collapse

"Why did the Black Death trigger a long-term social and economic crisis?"

Physicians were powerless against the plague, often dying in numbers equal to or greater than their patients. Treatments were desperate improvisations of humoral and supernatural remedies: lancing and draining buboes with hot irons, applying live toads or plucked chickens to draw out poison, bloodletting until the patient fainted, and administering spoonfuls of crushed emeralds, Theriac, or vinegar. None of these measures halted the infection, and lancing often accelerated septic shock and fatal blood poisoning.

As the death toll escalated beyond the capacity of traditional churchyards, London authorities purchased new emergency cemetery grounds at East Smithfield and Spitalfields, excavating colossal mass burial trenches where hundreds of corpses were stacked in layers like cargo in a ship's hold. Between 1348 and 1350, an estimated 30% to 45% of England's population—nearly two million people—perished. Whole villages were abandoned, agricultural fields lay unharvested, and livestock roamed untended. The catastrophic demographic collapse produced severe labour shortages, empowering surviving serfs to demand higher wages and directly undermining the feudal economic order.

Master Knowledge Retrieval Quiz

Medieval Medicine in Britain • c1250–c1500 (30 Essential Recall Questions)

Standard Recall Protocol: Partner Quiz or Solo Practice. Test your rapid chronological, factual, and conceptual recall across all 5 enquiries. All questions are cross-referenced directly to paragraph locations in this textbook; zero answers are printed to maintain active retrieval integrity.

Enquiry 01: Supernatural & Religious Expla… pp. 2–3 • Ref [1.1]–[1.5]

Enquiry 02: Rational Explanations: Hippocr… pp. 4–5 • Ref [2.1]–[2.5]

Enquiry 03: Approaches to Prevention & Tr… pp. 6–7 • Ref [3.1]–[3.5]

Enquiry 04: Medical Care Providers & Mon… pp. 8–9 • Ref [4.1]–[4.5]

Enquiry 05: Case Study: Dealing with the B… pp. 10–11 • Ref [5.1]–[5.5]

Synoptic Synthesis & Historical Causation

pp. 2–11 • Whole-Era Recall

26. **[Synoptic]** Explain the single biggest obstacle to medical progress across this era.

27. **[Synoptic]** Which institution or technological breakthrough triggered the greatest shift in clinical practice?

28. **[Synoptic]** How did contemporary beliefs about the causes of disease influence the treatments administered?

29. **[Synoptic]** Assess the extent of continuity between the start and end of this chronological period.

30. **[Synoptic]** Identify one individual whose contributions laid the groundwork for subsequent breakthroughs.